3.2 Bronchial Asthma*

Assessment	Nursing Diagnosis	Plan / Expected Outcome	Nursing Interventions	Evaluation
Subjective • "My chest feels tight and I cannot breathe out." **Objective** • Expiratory wheeze • Prolonged expiration • RR 28/min, SpO_2 91%	**ACTUAL** Ineffective breathing pattern related to bronchoconstriction as evidenced by wheeze, prolonged expiration and tachypnea.	• RR 12–20/min and SpO_2 94% or above within 2 hours.	• Sit her up; leaning forward may help. • Give the inhaled short-acting bronchodilator as prescribed and judge the response. • Titrate oxygen to the target SpO_2. • Give the prescribed corticosteroid. • Stay with her and keep a calm manner.	• Considered met when the wheeze settles and SpO_2 reaches target. • No improvement after treatment is reported at once.
Subjective • "I use the inhaler only when I feel bad." **Objective** • Poor inhaler technique • Several exacerbations this year	**ACTUAL** Ineffective health self-management related to insufficient knowledge of the therapeutic regimen as evidenced by incorrect inhaler use and frequent exacerbations.	• Before discharge, uses the inhaler correctly and can tell reliever from preventer.	• Teach inhaler and spacer technique and watch her repeat it. • Explain what the reliever does and what the preventer does. • Help her identify her own triggers, such as dust, smoke and cold air. • Give her a written asthma action plan. • Remind her to rinse her mouth after the inhaled steroid.	• Goal reached when technique and explanation are both correct.
Subjective • "I wait until the attack is bad before going to hospital." **Objective** • Uses reliever more than 3 times a week • Past severe exacerbations	**RISK** Risk for physical injury as evidenced by risk factors: delayed treatment of exacerbations and overuse of reliever medication.	• Can say when to follow the action plan and when to seek emergency care.	• Teach the warning signs: rising reliever use, waking at night, difficulty speaking. • Go through each step of the action plan. • Tell her to come to the emergency department if the reliever stops working.	• Met when the warning signs and actions are stated correctly.

* **References for this care plan:** [25], [35], [37], [38], [48]. Full entries are listed in the References section of the CPMN book.